

Lip and tongue frenula

When correction may help, what V-Y frenuloplasty involves and how to care for the wound.

This information is for adults, parents and their children. We discuss which frenulum needs treatment and what improvement is realistically achievable. Assessment, technique and aftercare for infants require a separate individual plan.

A normal fold can restrict movement

Labial frenula connect the inside of the lip to the mucosa around the jaw. The lingual frenulum connects the underside of the tongue to the floor of the mouth. Their attachment, length and flexibility vary. **A visible or prominent frenulum alone is not a reason for surgery.** Symptoms and function determine whether treatment is appropriate.

Lip frenulum

An unfavourable attachment may pull on the gum, make cleaning difficult or interfere with lip movement. For a persistent gap between the upper front teeth, correction may be part of an orthodontic plan. We assess whether the frenulum actually contributes to the problem.

Tongue frenulum

A short or tight frenulum may limit lifting or moving the tongue. Relevant symptoms can include difficulty eating, cleaning the mouth or making certain speech movements. Speech problems require assessment by a speech and language therapist.

Choosing the right time

A gap between the front teeth can be normal while permanent teeth are developing. Timing of lip frenulum surgery is coordinated with orthodontics; further dental development or planned space closure is often awaited. **Surgery alone does not close a tooth gap.**

Infants with feeding difficulties: Feeding technique, weight gain and other causes should first be assessed by paediatrics and qualified breastfeeding/feeding support. Tongue-tie surgery is considered only for significant functional restriction and persistent problems despite support. A prominent lip frenulum or concern about possible future speech problems alone does not justify preventive surgery.

Our usual technique: V-Y frenuloplasty

V-Y frenuloplasty releases and repositions mucosa to lengthen restrictive tissue. A **V-shaped area is closed with a Y-shaped suture line**. The aim is to reduce pulling and allow more movement. The precise incision depends on which frenulum is involved and the tissue available.

1. **Plan:** We assess function, tissue tension and nearby structures. Where needed, we coordinate with dentistry, orthodontics, speech therapy or paediatrics.
2. **Anaesthetise:** Surgery starts only once anaesthesia is effective. With local anaesthesia, pressure and touch can still be felt. Tell us immediately if anything hurts.
3. **Release and reposition:** Restrictive tissue is released while protecting surrounding structures, and the mucosa is moved according to the individual findings.
4. **Close and check:** We control bleeding, check movement and close the wound with fine stitches with as little tension as possible.

Treatment is usually an outpatient procedure. Time required depends on the extent, anaesthesia and cooperation. Depending on the material, stitches are **removed after 1–2 weeks** or dissolve. We arrange the review individually.

What improvement can be expected?

The goal is better function. Normal speech immediately after surgery, a particular range of tongue movement or lasting tooth-gap closure cannot be guaranteed. Established movement patterns may need further therapy after healing. The scar inside the mouth may tighten again; reattachment or repeat surgery is possible.

What are the alternatives?

Depending on the findings, observation, oral hygiene advice, speech therapy, guided functional exercises or orthodontic treatment may be appropriate. These do not always sufficiently relieve a genuinely tight band. Surgical alternatives include simple division, removal of restrictive tissue or another form of local tissue rearrangement. We explain which options suit your situation.

Without surgery, pulling or restricted movement may persist. Not every frenulum becomes more troublesome; observation may be appropriate when there are no significant symptoms.

Clarifying costs beforehand

Charges and insurance coverage depend on medical indication, treatment complexity and insurance. Before treatment, we clarify which services are covered and whether **individually agreed private fees** apply. Anaesthesia and comfort options are discussed separately.

Possible risks and complications

Even minor surgery may cause problems or require additional treatment. Individual risk depends on the findings, previous surgery, general health, medication and wound care.

- **Wound and healing:** pain, swelling, bruising, bleeding, infection, wound opening and delayed healing. Further treatment to stop bleeding or replace stitches may be needed.
- **Scarring and function:** tight scarring, reattachment or persistent restriction; rarely, movement can worsen. Further therapy or revision surgery may be necessary.
- **Lip frenulum:** damage to nearby mucosa or gums, gum recession or a change in contour. A gap between the teeth may remain or recur.
- **Tongue frenulum:** injury to blood vessels, salivary ducts or nerves. This may cause blocked salivary flow or a salivary cyst, and temporary or permanent changes in sensation, taste or movement.
- **Rare serious complications:** heavy bleeding or swelling in the floor of the mouth affecting swallowing or breathing. Call 112 for rapidly increasing swelling or breathing difficulty.
- **Anaesthesia and medicines:** intolerance, allergic reactions, circulatory or breathing problems. Sedation and general anaesthesia require an additional anaesthetic consultation.

Wound care and daily activities

Until numbness has worn off: do not chew or drink anything hot. Supervise children so that they do not bite their lip or tongue. Afterwards, choose soft, lukewarm food and drink enough fluids. Infants receive an individual feeding plan.

Clean the teeth gently with a soft brush. Do not scrub the stitches or touch the wound with your fingers. Avoid vigorous rinsing or spitting on the day of surgery; use rinses and medicines only as instructed. Do not smoke or vape, and avoid alcohol during early healing. Rest from physical exertion for the first few days; resume sport and heavier activity according to recovery and our advice.

Move according to your plan; do not force stretching. We tailor tongue or lip exercises to the findings, stitches and healing, with speech therapy where appropriate. Do not pull the wound apart or reopen it. There is no universal stretching routine for everyone, particularly infants.

Anaesthesia and comfort options

Method	Description	Cost
Local anaesthesia Standard	You remain awake and feel pressure, not pain. If you feel pain, more anaesthetic is given immediately. You may use public transport home.	Statutory insurance benefit or included in the treatment package
Oral sedation (Tavor) Comfort option	A tablet about 60 minutes beforehand. It reduces stress without inducing sleep. Vital signs are monitored. An escort is needed; no driving for 24 hours. Collection from the practice is mandatory.	Approx. €80 Out-of-pocket
IV sedation Comfort option	A professional anaesthesia team provides medication at the sleep–wake boundary. You remain cooperative and usually have little memory. Recovery room and vital-sign monitoring afterwards. Tuesdays only. No food from 10 pm and no drinks from 6 am. Collection from the practice is mandatory.	Approx. €230 Out-of-pocket
General anaesthesia Comfort option	Deep sleep, with a breathing tube through the nose into the airway. No awareness of surgery. Recovery room and vital-sign monitoring afterwards. Tuesdays only. Same fasting instructions as IV sedation. Collection from the practice is mandatory.	Approx. €450 Out-of-pocket

Local anaesthesia is the standard and is sufficient for most patients. Sedation is an optional comfort service paid out of pocket. With every method, the anaesthetic is fully effective before we begin so that you do not feel pain.

For children and adolescents, the method is chosen individually according to age, health and cooperation. Tavor is not a routine recommendation for minors. Your agreed written fasting and medication instructions take priority. Medically necessary general anaesthesia and possible insurance coverage are assessed separately.

What we need to know before your appointment

- Provide your medication list, allergies, bleeding disorders, previous excessive bleeding and earlier operations. Do not stop blood thinners, including prescribed aspirin, yourself.
- Tell us about diabetes, immunosuppression, relevant heart/lung disease and possible pregnancy.
- Bring orthodontic or speech therapy findings and the agreed treatment goal. For minors, clarify parental responsibility and accompaniment.
- With local anaesthesia alone, a light meal beforehand is usually appropriate. For sedation/general anaesthesia, follow only your individually agreed fasting instructions.

Getting home: Do not drive a car, ride a bicycle or motorcycle after local anaesthesia; public transport is generally possible. Minors must be accompanied. After sedation or general anaesthesia, collection from the practice and supervision are required. For at least 24 hours, do not drive, operate machinery, drink alcohol or make important decisions.

Managing pain

Children and adolescents receive an individual plan based on age and weight. The table below is the adult practice standard and applies only after individual approval. Health conditions, body weight and other medicines may require lower doses or different medication.

Medicine	Dose if suitable	Note
Ibuprofen 600 mg	Every 6–8 hours, max. 3 doses/day	Take with food
Paracetamol 500–1000 mg	Alternatively/additionally every 6 hours	Max. 4 g/day; not suitable for all body weights
Novalgin drops	Up to 30 drops 4 times/day, only if prescribed	Only at 500 mg/ml and 20 drops/ml
No additional aspirin/ASA	Avoid as a painkiller	Do not independently stop prescribed long-term aspirin

If fever, chills, sore throat or painful mucosal sores develop while taking Novalgin/metamizole: **stop it immediately and seek urgent medical assessment.** Do not add paracetamol-containing combination products without advice.

Healing and reviews

- **First few days:** soreness, swelling and unfamiliar movement are possible. Symptoms should gradually improve.
- **After about 1–2 weeks:** wound and function review, with suture removal where required. A whitish coating can be part of healing; do not scrape it off.
- **Following weeks:** the scar matures and movement is reassessed. Speech therapy exercises or orthodontics continue according to the agreed plan.

Contact us promptly for warning signs

Persistent bleeding, clearly worsening pain or swelling, fever, pus or inadequate fluid intake: practice **06151 786540**.

After surgery outside practice hours: **+49 151 10437450**. If unavailable: dental emergency service **01805 60 70 11**.

For a life-threatening emergency, breathing difficulty, severe difficulty swallowing or rapidly increasing swelling of the floor of the mouth, neck or tongue, **call 112 immediately**.

Frequently asked questions

Does surgery make the tongue longer?

The tongue itself is not lengthened. Restrictive tissue is released or lengthened so that the existing tongue can move more freely.

Will a speech problem disappear on its own afterwards?

This is not certain. Together with speech therapy, we assess whether the frenulum contributes to the difficulty. Exercises or further therapy may still be needed after surgery.

Does a prominent lip frenulum always need removing?

No. Without problematic pulling or a clear functional or orthodontic indication, surgery is often unnecessary.

Are mouthwash and antibiotics always necessary?

No. They are prescribed only when indicated. Not all mouthwashes are suitable for young children; do not use them without advice.

What if there is bleeding or my child will not drink?

Contact the practice promptly for persistent bleeding or clearly inadequate fluid intake. If cooperation is safe, gentle pressure with clean gauze on an accessible wound may help. Never leave a young child unattended with loose gauze in the mouth. Do not press blindly under the tongue or reach deep into the mouth.

Your treatment checklist

- The affected frenulum, functional problem and realistic goal are clear.
- Alternatives, risks, anaesthesia and costs have been discussed.
- Medication, accompaniment and any fasting instructions are agreed.
- The review, suture care and start of any exercises are planned.
- You know how to contact us for warning signs.



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Clinical sources: AAPD: Management of the Frenulum (2022; Reference Manual 2025); AAP: Ankyloglossia and breastfeeding (2024); Evelina London: Tongue-tie surgery; East Sussex Healthcare NHS: Frenectomy; V-Y plasty for abnormal frenal attachment (case report, 2024). Accessed 24 September 2026. Practice protocol: MKG im Carree. This information supports your individual consent discussion.